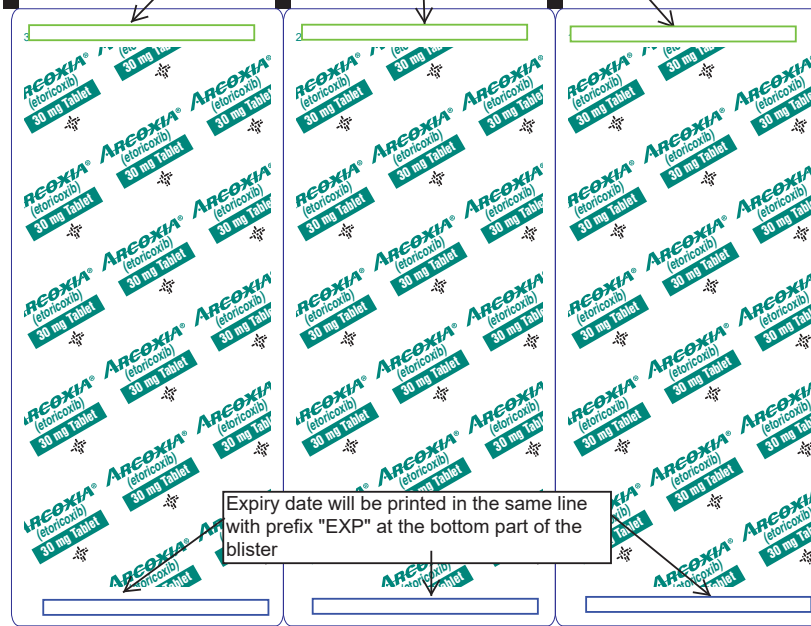


Lot number will be printed in the same line
with prefix "LOT" at the top part of the blister



Expiry date will be printed in the same line
with prefix "EXP" at the bottom part of the
blister

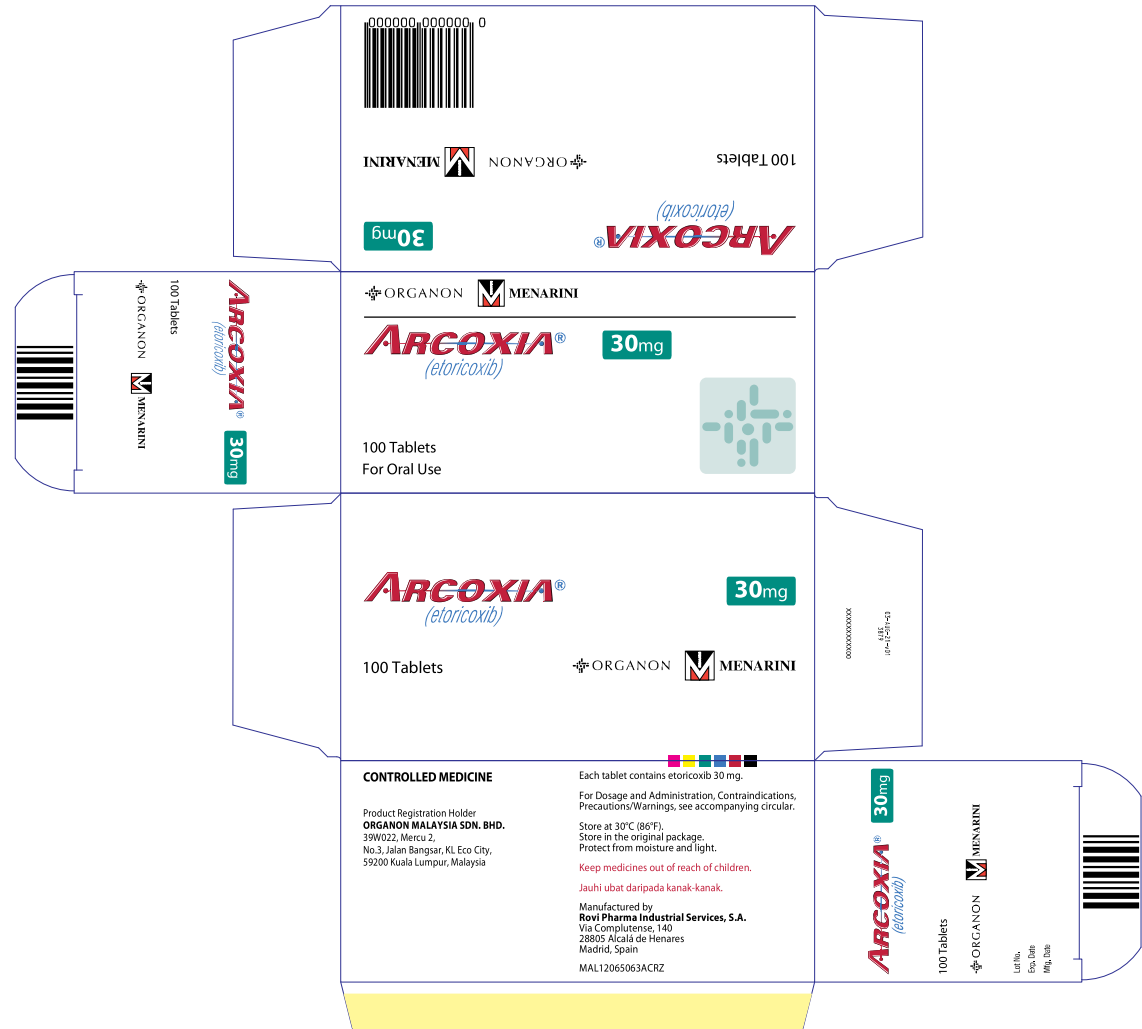


TRADE CARTON 30'S



The carton design for pack sizes 30 tabs, 10 tabs and 100 tabs are the same. There will be slight differences in the box dimensions.

TRADE CARTON 100'S



The carton design for pack sizes 30 tabs, 10 tabs and 100 tabs are the same. There will be slight differences in the box dimensions.

LOCAL PRODUCT CIRCULAR

Tablets
ARCOXIA®
(etoricoxib)

I. THERAPEUTIC CLASS

ARCOXIA® (etoricoxib) is a member of a class of arthritis/analgesia medications called Coxibs. ARCOXIA is a highly selective inhibitor of cyclooxygenase-2 (COX-2).

II. COMPOSITION

IIa. Active Ingredients

Each tablet of ARCOXIA for oral administration contains either 30, 60, 90 or 120 mg of etoricoxib.

IIb. Inactive Ingredients

Each tablet contains calcium hydrogen phosphate (anhydrous), carnauba wax, croscarmellose sodium, hypromellose, lactose monohydrate, magnesium stearate, microcrystalline cellulose, titanium dioxide, and glycerol triacetate. The 30 mg, 60 mg and 120 mg tablets also contain yellow ferric oxide and FD&C Blue #2 (indigo carmine lake).

III. INDICATIONS

ARCOXIA is indicated for:

- Acute and chronic treatment of the signs and symptoms of osteoarthritis (OA) and rheumatoid arthritis (RA)
- Treatment of ankylosing spondylitis (AS)
- Treatment of acute gouty arthritis
- Chronic low back pain (30 mg and 60 mg only)
- Treatment of acute pain, including that related to primary dysmenorrhoea and minor dental procedures.

The decision to prescribe a selective COX-2 inhibitor should be based on an assessment of the individual patient's overall risks (see PRECAUTIONS).

IV. DOSAGE AND ADMINISTRATION

ARCOXIA is administered orally. ARCOXIA may be taken with or without food. ARCOXIA should be administered for the shortest duration possible and the lowest effective daily dose should be used.

Osteoarthritis

The recommended dose is 30 mg or 60 mg once daily.

Rheumatoid Arthritis

The recommended dose is 60 mg or 90 mg once daily. The minimum effective daily dose is 60 mg once daily. In some patients, 90 mg once daily may provide increased therapeutic benefit.

Ankylosing Spondylitis

The recommended dose is 60 mg or 90 mg once daily. The minimum effective daily dose is 60 mg once daily. In some patients, 90 mg once daily may provide increased therapeutic benefit.

Chronic low back pain

The recommended dose is 60 mg once daily

Acute Pain

In the following acute painful conditions, ARCOXIA should be used only for the acute symptomatic period, limited to a maximum of 8 days treatment:

Acute Gouty Arthritis

The recommended dose is 120 mg once daily.

Primary Dysmenorrhea

The recommended dose is 120 mg once daily.

Minor Dental Procedures

The recommended dose is 90 mg once daily.

Doses greater than those recommended for each indication have either not demonstrated additional efficacy or have not been studied. Therefore:

The dose for OA should not exceed 60 mg daily.

The dose for RA should not exceed 90 mg daily.

The dose for ankylosing spondylitis should not exceed 90 mg daily.

The dose for acute gout should not exceed 120 mg daily.

The dose for acute pain and primary dysmenorrhea should not exceed 120 mg daily.

The dose of chronic low back pain should not exceed 60 mg daily.

The dose for minor dental procedures should not exceed 90 mg daily.

As the cardiovascular risks of selective COX-2 inhibitors may increase with dose and duration of exposure, the shortest duration possible and the lowest effective daily dose should be used. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically (see PRECAUTIONS).

Elderly, Gender, Race

No dosage adjustment in ARCOXIA is necessary for the elderly or based on gender or race.

Hepatic Insufficiency

In patients with mild hepatic insufficiency (Child-Pugh score 5-6), a dose of 60 mg once daily should not be exceeded. In patients with moderate hepatic insufficiency (Child-Pugh score 7-9), the dose should be reduced; a dose of 60 mg *every other day* should not be exceeded, administration of 30 mg once daily can also be considered. There are no clinical or pharmacokinetic data in patients with severe hepatic insufficiency (Child-Pugh score >9). (see PRECAUTIONS).

Renal Insufficiency

In patients with advanced renal disease (creatinine clearance <30 mL/min), treatment with ARCOXIA is not recommended. No dosage adjustment is necessary for patients with lesser degrees of renal insufficiency (creatinine clearance ≥30 mL/min). (see PRECAUTIONS).

V. CLINICAL PHARMACOLOGY

IIIa. Mechanism of Action

ARCOXIA is a nonsteroidal anti-inflammatory drug (NSAID) that exhibits anti-inflammatory, analgesic, and antipyretic activities in animal models. ARCOXIA is a potent, orally active, highly selective cyclooxygenase-2 (COX-2) inhibitor within and above the clinical dose range. Two isoforms of cyclooxygenase have been identified: cyclooxygenase-1 (COX-1) and cyclooxygenase-2 (COX-2). COX-1 is responsible for prostaglandin-mediated normal physiologic functions such as gastric cytoprotection and platelet aggregation. Inhibition of COX-1 by nonselective NSAIDs has been associated with gastric damage and platelet inhibition. COX-2 has been shown to be primarily responsible for the synthesis of prostanoid mediators of pain, inflammation, and fever. Selective inhibition of COX-2 by etoricoxib decreases these clinical signs and symptoms with decreased GI toxicity and without effects on platelet function.

Across clinical pharmacology studies, ARCOXIA produced dose-dependent inhibition of COX-2 without inhibition of COX-1 at doses up to 150 mg daily.

The influence on gastroprotective COX-1 activity was also assessed in a clinical study where prostaglandin synthesis was measured in gastric biopsy samples from subjects administered either ARCOXIA 120 mg daily, naproxen 500 mg twice daily, or placebo. ARCOXIA did not inhibit gastric prostaglandin synthesis as compared to placebo. In contrast, naproxen inhibited gastric prostaglandin synthesis by approximately 80% compared with placebo. These data further support the COX-2 selectivity of ARCOXIA.

Platelet Function

Multiple doses of ARCOXIA up to 150 mg administered daily up to nine days had no effect on bleeding time relative to placebo. Similarly, bleeding time was not altered in a single dose study with ARCOXIA 250 or 500 mg. There was no inhibition of *ex vivo* arachidonic acid- or collagen-induced platelet aggregation at steady state with doses of ARCOXIA up to 150 mg. These findings are consistent with the COX-2 selectivity of etoricoxib.

<i>IIIb. Pharmacokinetics</i>
<i>IIIb-1. Absorption</i>

In studies specifically designated to measure the onset of action of etoricoxib, the onset of action occurred as early as 24 minutes after dosing.

Orally administered etoricoxib is well absorbed. The mean oral bioavailability is approximately 100%. Following 120 mg once-daily dosing to steady state, the peak plasma concentration (geometric mean C_{max} = 3.6 mcg/mL) was observed at approximately 1 hour (T_{max}) after administration to fasted adults. The geometric mean AUC_{0-24hr} was 37.8 mcg•hr/mL. The pharmacokinetics of etoricoxib are linear across the clinical dose range.

A standard meal had no clinically meaningful effect on the extent or rate of absorption of a dose of etoricoxib 120 mg. In clinical trials, etoricoxib was administered without regard to food.

The pharmacokinetics of etoricoxib in 12 healthy subjects were similar (comparable AUC, $C_{\max}$ within approximately 20%) when administered alone, with a magnesium/aluminium hydroxide antacid, or a calcium carbonate antacid (approximately 50 mEq acid-neutralizing capacity).

IIIb-2. Distribution

Etoricoxib is approximately 92% bound to human plasma protein over the range of concentrations of 0.05 to 5 mcg/mL. The volume of distribution at steady state (V_{dss}) is approximately 120 L in humans.

Etoricoxib crosses the placenta in rats and rabbits, and the blood-brain barrier in rats.

IIIb-3. Metabolism

Etoricoxib is extensively metabolized with <1% of a dose recovered in urine as the parent drug. The major route of metabolism to form the 6'-hydroxymethyl derivative is catalyzed by cytochrome P450 (CYP) enzymes.

Five metabolites have been identified in man. The principal metabolite is the 6'-carboxylic acid derivative of etoricoxib formed by further oxidation of the 6'-hydroxymethyl derivative. These principal metabolites either demonstrate no measurable activity or are only weakly active as COX-2 inhibitors. None of these metabolites inhibit COX-1.

IIIb-4. Elimination

Following administration of a single 25 mg radiolabeled intravenous dose of etoricoxib to healthy subjects, 70% of radioactivity was recovered in urine and 20% in feces, mostly as metabolites. Less than 2% was recovered as unchanged drug.

Elimination of etoricoxib occurs almost exclusively through metabolism followed by renal excretion. Steady state concentrations of etoricoxib are reached within seven days of once-daily administration of 120 mg, with an accumulation ratio of approximately 2, corresponding to an accumulation half-life of approximately 22 hours. The plasma clearance is estimated to be approximately 50 mL/min.

VI. CONTRAINDICATIONS

ARCOXIA is contraindicated in patients with:

Hypersensitivity to any component of this product.

Congestive heart failure (NYHA II-IV).

Established ischemic heart disease, peripheral arterial disease and/or cerebrovascular disease (including patients who have recently undergone coronary artery bypass graft surgery or angioplasty).

Patients with hypertension whose blood pressure has not been adequately controlled.

Pregnancy.

VII. PRECAUTIONS

Clinical trials suggest that the selective COX-2 inhibitor class of drugs may be associated with an increased risk of thrombotic events (especially MI and stroke), relative to placebo and some NSAIDs (naproxen). As the cardiovascular risks of selective COX-2 inhibitors may increase with dose and duration of exposure, the shortest duration possible and the lowest effective daily dose should be used. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically.

Patients with significant risk factors for cardiovascular events (e.g. hypertension, hyperlipidaemia, diabetes mellitus, smoking) should only be treated with etoricoxib after careful consideration.

Selective COX-2 inhibitors are not a substitute for aspirin for cardiovascular prophylaxis because of their lack of effect on platelets. Because etoricoxib, a member of this class, does not inhibit platelet aggregation, antiplatelet therapies should not be discontinued.

There is a further increase in the risk of gastrointestinal adverse effects (gastrointestinal ulceration or other gastrointestinal complications) for etoricoxib, other selective COX-2 inhibitors and NSAIDs, when taken concomitantly with acetylsalicylic acid (even at low doses). The relative difference in gastrointestinal safety between selective COX-2 inhibitors + acetylsalicylic acid vs. NSAIDs + acetylsalicylic acid has not been adequately evaluated in long-term clinical trials.

In patients with advanced renal disease, treatment with ARCOXIA is not recommended. Clinical experience in patients with estimated creatinine clearance of <30 mL/min is very limited. If therapy with ARCOXIA must be initiated in such patients, close monitoring of the patient's renal function is advisable.

Long-term administration of NSAIDs has resulted in renal papillary necrosis and other renal injury. Renal prostaglandins may play a compensatory role in the maintenance of renal perfusion. Therefore, under conditions of compromised renal perfusion, administration of

ARCOXIA may cause a reduction in prostaglandin formation and, secondarily, in renal blood flow, and thereby impair renal function. Patients at greatest risk of this response are those with pre-existing significantly impaired renal function, uncompensated heart failure, or cirrhosis. Monitoring of renal function in such patients should be considered.

Caution should be used when initiating treatment with ARCOXIA in patients with considerable dehydration. It is advisable to rehydrate patients prior to starting therapy with ARCOXIA.

As with other drugs known to inhibit prostaglandin synthesis, fluid retention, edema and hypertension have been observed in some patients taking ARCOXIA. The possibility of fluid retention, edema or hypertension should be taken into consideration when ARCOXIA is used in patients with pre-existing edema, hypertension, or heart failure. All Nonsteroidal Antiinflammatory Drugs (NSAIDs), including etoricoxib, can be associated with new onset or recurrent congestive heart failure. (See SIDE EFFECTS.). Etoricoxib may be associated with more frequent and severe hypertension than some other NSAIDs and selective COX-2 inhibitors, particularly at high doses. Therefore, special attention should be paid to blood pressure monitoring during treatment with etoricoxib. If blood pressure rises significantly, alternative treatment should be considered.

Physicians should be aware that individual patients may develop upper gastrointestinal (GI) ulcers/ulcer complications irrespective of treatment. Although the risk of GI toxicity is not eliminated with ARCOXIA, the results of the MEDAL Program demonstrate that in patients treated with ARCOXIA, the risk of GI toxicity with ARCOXIA 60 mg or 90 mg once daily is significantly less than with diclofenac 150 mg daily. In clinical studies with ibuprofen and naproxen, the risk of endoscopically detected upper GI ulcers was lower in patients treated with ARCOXIA 120 mg once daily than in patients treated with the non-selective NSAIDs. While the risk of endoscopically detected ulcers was low in patients treated with ARCOXIA 120 mg it was higher than in patients treated with placebo. Upper GI ulcers/ulcer complications have occurred in patients treated with ARCOXIA. These events can occur at any time during use and without warning symptoms. Independent of treatment, patients with a prior history of GI perforation, ulcers and bleeding (PUB) and patients greater than 65 years of age are known to be at a higher risk for a PUB.

Elevations of alanine aminotransferase (ALT) and/or aspartate aminotransferase (AST) (approximately three or more times the upper limit of normal) have been reported in approximately 1% of patients in clinical trials treated for up to one year with ARCOXIA 30, 60 and 90 mg daily. In active comparator portions of clinical trials, the incidence of elevated AST and/or ALT in patients treated with ARCOXIA 60 and 90 mg daily was similar to that of

patients treated with naproxen 1000 mg daily, but notably less than the incidence in the diclofenac 150 mg daily group. These elevations resolved in patients treated with ARCOXIA, with approximately half resolving while patients remained on therapy. In controlled clinical trials of ARCOXIA 30 mg daily versus ibuprofen 2400 mg daily or celecoxib 200 mg daily, the incidence of elevations of ALT or AST was similar.

A patient with symptoms and/or signs suggesting liver dysfunction, or in whom an abnormal liver function test has occurred, should be evaluated for persistently abnormal liver function tests. If persistently abnormal liver function tests (three times the upper limit of normal) are detected, ARCOXIA should be discontinued.

ARCOXIA should be used with caution in patients who have previously experienced acute asthmatic attacks, urticaria, or rhinitis, which were precipitated by salicylates or non-selective cyclooxygenase inhibitors. Since the pathophysiology of these reactions is unknown, physicians should weigh the potential benefits of prescribing ARCOXIA versus the potential risks.

When using etoricoxib in the elderly and in patients with renal, hepatic, or cardiac dysfunction, medically appropriate supervision should be maintained. If these patients deteriorate during treatment, appropriate measures should be taken, including discontinuation of therapy.

Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens-Johnson syndrome, and toxic epidermal necrolysis, have been reported very rarely in association with the use of NSAIDs and some selective COX-2 inhibitors during post-marketing surveillance (See SIDE EFFECTS). These serious events may occur without warning. Patients appear to be at highest risk for these reactions early in the course of therapy: the onset of the reaction occurring in the majority of cases within the first month of treatment. Serious hypersensitivity reactions (such as anaphylaxis and angioedema) have been reported in patients receiving etoricoxib (See SIDE EFFECTS). Some selective COX-2 inhibitors have been associated with an increased risk of skin reactions in patients with a history of any drug allergy. Etoricoxib should be discontinued at the first appearance of skin rash, mucosal lesions, or any other sign of hypersensitivity.

ARCOXIA may mask fever, which is a sign of infection. The physician should be aware of this when using ARCOXIA in patients being treated for infection.

VIII. WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAIDs

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAIDs therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

IX. PREGNANCY

The use of etoricoxib, as with any drug substance known to inhibit COX-2, is not recommended in women attempting to conceive. No clinical data on exposed pregnancies are available for etoricoxib. Studies in animals have shown reproductive toxicity. The potential for human risk in pregnancy is unknown. Etoricoxib, as with other medicinal products inhibiting prostaglandin synthesis, may cause uterine inertia and premature closure of the ductus arteriosus during the last trimester. Cases of fetal renal dysfunction that have resulted in reduction of amniotic fluid volume (oligohydramnios) have been reported in pregnant women treated with non-steroidal anti-inflammatory drugs (NSAIDs) at 20 weeks of gestation or later. In some cases, this may result in neonatal renal dysfunction. Such effects may occur shortly after NSAID treatment initiation; oligohydramnios is often reversible after treatment discontinuation. Etoricoxib is contraindicated in pregnancy. If a woman becomes pregnant during treatment, etoricoxib should be discontinued.

X. NURSING MOTHERS

Etoricoxib is excreted in the milk of lactating rats. It is not known whether this drug is excreted in human milk. Because many drugs are excreted in human milk and because of the possible adverse effects of drugs that inhibit prostaglandin synthesis on nursing infants, a decision should be made whether to discontinue nursing or to discontinue the drug, taking into account the importance of the drug to the mother.

XI. PEDIATRIC USE

Safety and effectiveness of etoricoxib in pediatric patients have not been established.

XII. USE IN THE ELDERLY

Pharmacokinetics in the elderly (65 years of age and older) are similar to those in the young. In clinical studies, a higher incidence of adverse experiences was seen in older patients compared to younger patients; the relative differences between etoricoxib and control groups were similar in the elderly and the young. Greater sensitivity of some older individuals cannot be ruled out.

XIII. DRUG INTERACTIONS

Warfarin: In subjects stabilized on chronic warfarin therapy, the administration of ARCOXIA 120 mg daily was associated with an approximate 13% increase in prothrombin time International Normalized Ratio (INR). Standard monitoring of INR values should be conducted when therapy with ARCOXIA is initiated or changed, particularly in the first few days, in patients receiving warfarin or similar agents.

Rifampin: Co-administration of ARCOXIA with rifampin, a potent inducer of hepatic metabolism, produced a 65% decrease in etoricoxib plasma area under the curve (AUC). This interaction should be considered when ARCOXIA is co-administered with rifampin.

Methotrexate: Two studies investigated the effects of ARCOXIA 60, 90 or 120 mg administered once daily for seven days in patients receiving once-weekly methotrexate doses of 7.5 to 20 mg for rheumatoid arthritis. ARCOXIA at 60 and 90 mg had no effect on methotrexate plasma concentrations (as measured by AUC) or renal clearance. In one study, ARCOXIA 120 mg had no effect on methotrexate plasma concentrations (as measured by AUC) or renal clearance. In the other study, ARCOXIA 120 mg increased methotrexate plasma concentrations by 28% (as measured by AUC) and reduced renal clearance of methotrexate by 13%. Monitoring for methotrexate-related toxicity should be considered when ARCOXIA at doses greater than 90 mg daily and methotrexate are administered concomitantly.

Diuretics, Angiotensin Converting Enzyme (ACE) Inhibitors and Angiotensin II

Antagonists (AIIAs): Reports suggest that NSAIDs including selective COX-2 inhibitors may diminish the antihypertensive effect of diuretics, ACE inhibitors and AIIAs. This interaction should be given consideration in patients taking ARCOXIA concomitantly with these products.

In some patients with compromised renal function (e.g., elderly patients or patients who are volume-depleted, including those on diuretic therapy) who are being treated with non-steroidal anti-inflammatory drugs, including selective COX-2 inhibitors, the co-administration of ACE inhibitors or AIIAs may result in a further deterioration of renal function, including possible acute renal failure. These effects are usually reversible. Therefore, the combination should be administered with caution, especially in the elderly.

Lithium: Reports suggest that non-selective NSAIDs and selective COX-2 inhibitors may increase plasma lithium levels. This interaction should be given consideration in patients taking ARCOXIA concomitantly with lithium.

Aspirin: ARCOXIA can be used concomitantly with low-dose aspirin at doses for cardiovascular prophylaxis. At steady state, etoricoxib 120 mg once daily had no effect on the anti-platelet activity of low-dose aspirin (81 mg once daily). However, concomitant administration of low-dose aspirin with ARCOXIA increases the rate of GI ulceration or other complications compared to use of ARCOXIA alone. (see PRECAUTIONS).

Oral Contraceptives: ARCOXIA 60mg given concomitantly with an oral contraceptive containing 35 mcg ethinyl estradiol (EE) and 0.5 to 1 mg norethindrone for 21 days increased the steady state AUC_{0-24hr} of EE by 37%. ARCOXIA 120 mg given with the same oral contraceptive either concomitantly or separated by 12 hours, increased the steady state AUC_{0-24hr} of EE by 50 to 60%. This increase in EE concentration should be considered when selecting an appropriate oral contraceptive for use with ARCOXIA. An increase in EE exposure can increase the incidence of adverse events associated with oral contraceptives (e.g., venous thrombo-embolic events in women at risk).

Hormone Replacement Therapy: Administration of ARCOXIA 120 mg with hormone replacement therapy consisting of conjugated estrogens (0.625 mg PREMARIN®) for 28 days, increased the mean steady state AUC_{0-24hr} of unconjugated estrone (41%), equilin (76%) and 17- β -estradiol (22%). The effect of the recommended chronic doses of ARCOXIA (30, 60 and 90 mg) has not been studied. The effects of ARCOXIA 120mg on the exposure (AUC_{0-24hr}) to these estrogenic components of PREMARIN® were less than half of those observed when PREMARIN® was administered alone and the dose was increased from 0.625 to 1.25 mg. The clinical significance of these increases is unknown, and higher doses of PREMARIN® were not studied in combination with ARCOXIA. These increases in estrogenic concentration should be taken into consideration when selecting post-menopausal hormone therapy for use with ARCOXIA.

Other: In drug-interaction studies, ARCOXIA did not have clinically important effects on the pharmacokinetics of prednisone/prednisolone or digoxin.

Antacids and ketoconazole (a potent inhibitor of CYP3A4) did not have clinically important effects on the pharmacokinetics of ARCOXIA.

XIV. SIDE EFFECTS

In clinical trials, ARCOXIA was evaluated for safety in 9295 individuals, including 5774 patients with OA, RA or chronic low back pain (approximately 600 patients with OA or RA were treated for one year or longer).

The following drug-related adverse experiences were reported in clinical studies in patients with OA, RA, or chronic low back pain treated for up to 12 weeks. These occurred in $\geq 1\%$ of patients treated with ARCOXIA and at an incidence greater than placebo: asthenia/fatigue, dizziness, lower extremity edema, hypertension, dyspepsia, heartburn, nausea, headache, ALT increased, AST increased.

The adverse experience profile was similar in patients with OA or RA treated with ARCOXIA for one year or longer.

In the MEDAL Study, an endpoint driven CV outcomes trial involving 23,504 patients, the safety of ARCOXIA 60 or 90 mg daily was compared to diclofenac 150 mg daily in patients with OA or RA (mean duration of treatment was 20 months). In this large trial, only serious adverse events and discontinuations due to any adverse events were recorded. The rates of confirmed thrombotic cardiovascular serious adverse events were similar between ARCOXIA and diclofenac. The incidence of discontinuations for hypertension-related adverse events was less than 3% in each treatment group; however, ARCOXIA 60 and 90 mg demonstrated significantly higher rates of discontinuations for these events than diclofenac. The incidence of congestive heart failure adverse events (discontinuations and serious events) and the incidence of discontinuations due to edema occurred at similar rates on ARCOXIA 60 mg compared to diclofenac, however, the incidences for these events were higher for ARCOXIA 90 mg compared to diclofenac. The incidence of discontinuations due to atrial fibrillation was higher for etoricoxib compared to diclofenac.

The EDGE and EDGE II studies compared the GI tolerability of etoricoxib 90 mg daily (1.5 to 3 times the doses recommended for OA) and diclofenac 150 mg daily in 7111 patients with OA (EDGE Study; mean duration of treatment 9 months) and 4086 patients with RA (EDGE II; mean duration of treatment 19 months). In each of these studies, the adverse experience profile on ARCOXIA was generally similar to that reported in the phase IIb/III placebo-controlled clinical studies; however, hypertension and edema-related adverse experiences occurred at a higher rate on etoricoxib 90 mg than on diclofenac 150 mg daily. The rate of confirmed thrombotic cardiovascular serious adverse events occurring in the two treatment groups was similar.

In a combined analysis of phase IIb to V clinical studies of 4 weeks duration or longer (excluding the MEDAL PROGRAM Studies), there was no discernible difference in the rate of confirmed thrombotic cardiovascular serious adverse events between patients receiving etoricoxib ≥ 30 mg or non-naproxen NSAIDs. The rate of these events was higher in patients receiving etoricoxib compared with those receiving naproxen 500 mg twice daily.

In a clinical study for ankylosing spondylitis, patients were treated with ARCOXIA 90 mg once daily for up to 1 year (N=126). In another clinical study for ankylosing spondylitis (N=857), patients were treated with ARCOXIA 60 mg or 90 mg once daily for up to 26 weeks. The adverse experience profile in these studies was generally similar to that reported in chronic

studies in OA, RA and chronic low back pain.

In a clinical study for acute gouty arthritis, patients were treated with ARCOXIA 120 mg once daily for eight days. The adverse experience profile in this study was generally similar to that reported in the combined OA, RA, and chronic low back pain studies.

In initial clinical studies for acute analgesia, patients were treated with ARCOXIA 120 mg once daily for one to seven days. The adverse experience profile in these studies was generally similar to that reported in the combined OA, RA, and chronic low back pain studies.

In the combined studies for acute post-operative dental pain, the incidence of post-dental extraction alveolitis (dry socket) reported in patients treated with ARCOXIA was similar to that of patients treated with active comparators.

Post-marketing experience

The following adverse reactions have been reported in post-marketing experience:

Blood and lymphatic system disorders: thrombocytopenia.

Immune system disorders: hypersensitivity reactions, anaphylactic/anaphylactoid reactions including shock.

Metabolism and nutrition disorders: hyperkalemia.

Psychiatric disorders: anxiety, insomnia, confusion, hallucinations, depression, restlessness.

Nervous system disorders: dysgeusia, somnolence, intracranial hemorrhage.

Eye disorders: blurred vision.

Cardiac disorders: congestive heart failure, palpitations, angina, arrhythmia.

Vascular disorders: hypertensive crisis, deep vein thrombosis.

Respiratory, thoracic and mediastinal disorders: bronchospasm, pulmonary embolism.

Gastrointestinal disorders: abdominal pain, oral ulcers, peptic ulcers including perforation and bleeding (mainly in elderly patients), vomiting, diarrhea.

Hepatobiliary disorders: hepatitis, jaundice, hepatic failure.

Skin and subcutaneous tissue disorders: angioedema, pruritus, erythema, rash, Stevens-Johnson syndrome, toxic epidermal necrolysis, urticaria, fixed drug eruption.

Renal and urinary disorders: renal insufficiency, including renal failure (see PRECAUTIONS).

XV. OVERDOSAGE

In clinical studies, administration of ARCOXIA at single doses up to 500 mg and multiple doses up to 150 mg/day for 21 days did not result in significant toxicity. There have been reports of acute overdosage with etoricoxib, although adverse experiences were not reported in the majority of cases. The most frequently observed adverse experience were consistent with the safety profile for etoricoxib. (e.g. gastrointestinal events, renovascular events).

In the event of overdose, it is reasonable to employ the usual supportive measures, e.g., remove unabsorbed material from the gastrointestinal tract, employ clinical monitoring, and institute supportive therapy, if required.

Etoricoxib is not dialyzable by hemodialysis; it is not known whether etoricoxib is dialyzable by peritoneal dialysis.

XVI. AVAILABILITY

ARCOXIA 30mg: Available in blister packs of 10's, 30's & 100's per pack

ARCOXIA 60mg: Available in blister packs of 10's & 100's per pack

ARCOXIA 90mg: Available in blister packs of 10's, 30's & 100's per pack

ARCOXIA 120mg: Available in blister packs of 30's per pack

Not all pack sizes may be marketed

XVII. PHYSICAL APPEARANCE

ARCOXIA 30mg: Blue-green apple-shape biconvex film coated tablets debossed ACX 30 on one side and 101 on the other side

ARCOXIA 60mg: Dark green apple-shape biconvex film coated tablets debossed ARCOXIA 60 on one side and 200 on the other side

ARCOXIA 90mg: White apple-shape biconvex film coated tablets debossed ARCOXIA 90 on one side and 202 on the other side

ARCOXIA 120mg: Pale green apple-shape biconvex film coated tablets debossed ARCOXIA 120 on one side and 204 on the other side

XVIII. STORAGE

Store at 30 ° C (86 ° F). Store in the original package. Protect from moisture and light.

XIX. SHELF LIFE

Please refer to the expiry date on the outer carton.

XX. MANUFACTURER

Rovi Pharma Industrial Services,
S.A.Vía Complutense, 140
28805 Alcalá de
HenaresMadrid,
Spain

XXI. PRODUCT REGISTRATION HOLDER

Organon Malaysia Sdn. Bhd.
39W022, Mercu 2,
No.3, Jalan Bangsar, KL Eco City,
59200 Kuala Lumpur, Malaysia

Date of Revision: 11 October 2023

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ARCOXIA® TABLET

Etoricoxib (30 mg, 60 mg, 90 mg, 120 mg)

What is in this leaflet

1. What Arcoxia tablet is used for
2. How Arcoxia tablet works
3. Before you take Arcoxia tablet
4. How to take Arcoxia tablet
5. While you are taking Arcoxia tablet
6. Side effects
7. Storage and Disposal of Arcoxia tablet
8. Product description
9. Manufacturer and Product Registration Holder
10. Date of Revision

What Arcoxia tablet is used for

Your doctor has prescribed Arcoxia tablet for:

- treatment of osteoarthritis
- treatment of rheumatoid arthritis
- treatment of ankylosing spondylitis
- chronic low back pain (30 mg and 60 mg only)
- treatment of acute pain after minor dental procedures
- treatment of menstrual pain
- treatment of gout attacks

Osteoarthritis is a joint disease. It results from the gradual breakdown of the cartilage that cushions the ends of the bones, resulting in pain, inflammation, tenderness, stiffness, and physical disability.

Rheumatoid arthritis is a chronic disease that causes pain, stiffness, swelling and loss of function in the joints and inflammation in other body organs.

Ankylosing spondylitis is an inflammatory disease of the spine and large joints.

Gout is a disorder characterized by sudden, recurring attacks of pain and inflammation in one or

more joints.

You may obtain further information from your doctor or pharmacist, who have more detailed information.

How Arcoxia tablet works

Arcoxia tablet is a member of a group of medicines called selective COX-2 inhibitors that are used to reduce pain and inflammation. Arcoxia tablet is not a narcotic.

Before you take Arcoxia tablet

- When you must not take it

Do not take Arcoxia tablet if you:

- are allergic to any of its ingredients
- have had heart failure, a heart attack, bypass surgery, chest pain, narrow or blocked arteries of the extremities, a stroke or mini stroke
- have high blood pressure that has not been controlled by treatment

- Before you start to take it

Tell your doctor about any medical problems or allergies you have now or have had, including:

- history of angina, heart attack or a blocked artery in your heart
- narrow or blocked arteries of the extremities
- kidney disease
- liver disease
- dehydration, for example by a prolonged bout of vomiting or diarrhea.
- have or have had ulceration, bleeding or perforation of your stomach or intestines
- heart failure
- high blood pressure
- swelling due to fluid retention
- an allergic reaction to aspirin or other anti-inflammatory

- medicines (commonly known as NSAIDs).
- a history of stroke or mini stroke
- conditions which increase your risk of coronary artery disease or atherosclerosis such as high blood pressure, diabetes, high cholesterol or smoking.
- being treated for an infection. Arcoxia tablet can mask or hide a fever, which is a sign of infection.
- have had asthma, nasal inflammation, and/or itching or hives caused by Aspirin or NSAIDs.
- suffering from dyspepsia (indigestion).

Etoricoxib may cause severe skin reactions. Symptoms may include skin rash, blisters and peeling of the skin. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Use in children

Arcoxia tablet has not been adequately studied in children. Therefore, Arcoxia tablet should not be given to children.

Use in the elderly

Arcoxia tablet works equally well in older and younger adult patients. Adverse experiences may occur at a higher incidence in older patients compared to younger patients. If you are elderly (i.e., over 65 years of age), your doctor will want to appropriately keep a check on you. No dosage adjustment is necessary for older patients.

Use in pregnancy and breastfeeding

Tell your doctor if you are:

- Arcoxia tablet may cause fetal harm and should not be

used by a woman who is attempting to become pregnant or becomes pregnant during treatment.

- Breastfeeding or plan to breastfeed.

- Taking other medicines

You should always tell your doctor about all medicines that you are taking or plan to take, including those obtained without a prescription.

Your doctor may want to check that your medicines are working properly if you are taking:

- warfarin (a blood thinner)
- rifampin (an antibiotic)
- diuretics (water tablets)
- ACE inhibitors and angiotensin receptor blockers (medicines used for high blood pressure and heart failure)
- lithium (a medicine used to treat a certain type of depression)
- birth control pills
- hormone replacement therapy
- methotrexate (a medicine used for suppressing the immune system).

Arcoxia tablet can be taken with low-dose aspirin. If you are currently taking low-dose aspirin for prevention of heart attack or stroke, you should not discontinue without consulting your doctor because Arcoxia tablet cannot substitute for aspirin for this purpose.

How to take Arcoxia tablet

- How much to take

Arcoxia tablet should be taken once a day. You may take Arcoxia tablet with or without food.

Your doctor will decide what dose of Arcoxia tablet you should take and how long you should take it.

For treatment of osteoarthritis:

The recommended dose is 30mg or 60 mg once a day.

For treatment of rheumatoid arthritis:

The recommended dose is 60 mg once a day, and may increase to 90 mg once a day if needed.

For treatment of ankylosing spondylitis:

The recommended dose is 60 mg once a day, and may increase to 90 mg once a day if needed.

Chronic low back pain:

The recommended dose is 60 mg once daily.

Acute pain conditions

The recommended dose is 90mg or 120mg once daily. Arcoxia should be used only for the acute period limited to a maximum of 8 days.

For the relief of gout attacks and the treatment of menstrual pain:

- The recommended dose is 120 mg once a day which should only be used for the acute painful period.

For the relief of pain after minor dental procedures

- The recommended dose is 90mg once a day.

Doses greater than those recommended for each condition have either not demonstrated additional efficacy or have not been studied. Therefore, the daily doses stated above for each condition should not be exceeded.

If you have mild or moderate liver disease, your doctor might prescribe you a lower dose or recommend that you do not take Arcoxia.

- When to take it

Use as directed by your doctor or pharmacist.

- How long to take it

Continue taking Arcoxia tablet for as long as your doctor recommends.

- If you forget to take it

Try to take Arcoxia tablet as prescribed. However, if you miss a dose, do not take an extra dose. Just resume your usual schedule the following day.

- If you take too much (overdose)

If you take more than the prescribed dosage, contact your doctor immediately.

While you are taking Arcoxia tablet

- Things you must do

Consult your doctor if:

- Any of the following symptoms appear or worsen: shortness of breath, chest pains or ankle swelling. Stop your treatment with Arcoxia tablet and consult a doctor, as soon as is practical.
- You have kidney, liver or heart disease, your doctor will want to appropriately keep a check on you.
- You develop any symptoms that could indicate a severe allergic reaction such as inability to breathe or a serious skin reaction, you must consult a doctor on an urgent basis.

Your doctor will want to discuss your treatment from time to time. It is important that you use the lowest dose that controls your pain.

Arcoxia tablet can increase blood pressure in some people, especially in high doses, and this could increase the risk of heart attacks and strokes. Your doctor will want to check your blood pressure from time to time, to make sure that it is safe to continue treatment.

- Things you must not do

Do not share Arcoxia tablet with anyone else; it was prescribed only for you.

Do not take Arcoxia tablet for longer than necessary. This is because the risk of heart attacks and strokes might increase after prolonged treatment, especially with high doses.

- Things to be careful of

Be careful driving or operating machinery until you know how Arcoxia affects you.

Arcoxia contains lactose

If you have been told by your doctor that you are unable to tolerate some sugars, contact your doctor before taking this medicinal product.

Side effects

Any medicine may have unintended or undesirable effects, so-called side effects. Like all prescription medicines, Arcoxia tablet may cause side effects.

Some of the side effects reported in the studies included:

- dizziness,
- swelling of the legs and/or feet,
- weakness and fatigue,
- high blood pressure,
- nausea,
- heartburn,
- upset stomach,
- headache
- dry socket (inflammation and pain after a tooth extraction)

Additionally, the following have been reported:

- allergic reactions (which maybe serious enough to require immediate medical attention) including swelling of the face, lips, tongue, and/or throat which may cause difficulty

inbreathing or swallowing, wheezing, rash, itching and hives;

- redness of the skin,
- severe skin reactions which may occur without warning;
- taste alteration,
- abnormal rhythm of the heart,
- heart failure,
- palpitations,
- feeling of tightness, pressure or heaviness in the chest,
- stomach pain,
- stomach ulcers that may become serious and may bleed and may occur at any time during use and without warning,
- vomiting,
- liver problems including liver failure, yellowing of the skin and eyes (jaundice),
- serious kidney problems,
- high levels of potassium in your blood,
- insomnia,
- anxiety,
- depression,
- restlessness,
- drowsiness,
- mouth ulcers,
- diarrhea,
- severe increase in blood pressure,
- confusion,
- hallucinations,
- platelets decreased,
- blurred vision,
- blood clots in the blood vessels of the lung and/or leg
- bleeding within the brain.

In clinical studies, the risk of developing ulcers while taking Arcoxia tablet was lower than with NSAIDs.

Other side effects may also occur rarely, and as with any prescription medicine, some side effects may be serious. Ask your doctor or pharmacist for more information. Both have a more complete list of side effects. Tell your doctor or pharmacist promptly about these or any other

unusual symptoms.

You may report any side effects or adverse drug reactions directly to the National Centre for Adverse Drug Reaction Monitoring by visiting the website npra.gov.my [Consumers → Reporting Side Effects to Medicines (ConSERF) or Vaccines (AEFI)].

Storage and Disposal

- Storage

Store at 30 °C (86 °F). Store in the original package. Protect from moisture and light.

Keep Arcoxia tablet and all medicines safely away from children.

Do not use this medicine after the month and year shown by the four numbers following EX (or EXP) on the container. The first two numbers indicate the month; the last two numbers indicate the year.

- Disposal

Medicines should not be disposed of via wastewater or household waste. Ask your pharmacist how to dispose of medicines no longer required. These measures will help to protect the environment.

Product Description

- What it looks like

- Arcoxia 30mg: Blue-green apple-shape biconvex film coated tablets debossed ACX 30 on one side and 101 on the other side
- Arcoxia 60mg: Dark green apple-shape biconvex film coated tablets debossed Arcoxia 60 on one side and 200 on the other side
- Arcoxia 90mg: White apple-shape biconvex film coated

- tablets debossed *ARCOXIA* 90 on one side and 202 on the other side
- Arcoxia 120mg: Pale green apple-shape biconvex film coated tablets debossed Arcoxia 120 on one side and 204 on the other side

- Ingredients

- Active ingredient: Etoricoxib

- Inactive ingredients: Calcium hydrogen phosphate anhydrous; microcrystalline cellulose; croscarmellose sodium; hypromellose; magnesium stearate; lactose monohydrate; titanium dioxide; glycerol triacetate and carnauba wax. The 30 mg, 60 mg and 120 mg tablets also contain yellow ferric oxide and FD&C Blue #2 (indigo carmine lake).

- MAL number Arcoxia 30 mg tablet: MAL12065063ACRZ

Arcoxia 60 mg tablet:
MAL12065082ACRZ

Arcoxia 90 mg tablet:
MAL12065065ACRZ

Arcoxia 120 mg tablet:
MAL12065084ACRZ

Product Manufacturer

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Product Registration Holder

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Date of revision

11/10/2023 (version 012023)

Serial number

NPRA (R1/2) 02012024/222
NPRA (R1/AI1) 24052023/012

ARCOXIA[®] TABLET

Risalah Maklumat Ubat untuk Pengguna (RiMUP)

Etoricoxib (30 mg, 60 mg, 90 mg, 120 mg)

Apakah yang ada pada risalah ini

1. Apakah kegunaan Arcoxia tablet
2. Bagaimana Arcoxia tablet berfungsi
3. Sebelum mengambil Arcoxia tablet
4. Cara mengambil Arcoxia tablet
5. Semasa mengambil Arcoxia tablet
6. Kesan-kesan sampingan
7. Cara penyimpanan dan pelupusan Arcoxia tablet
8. Maklumat lanjut
9. Pengilang dan Pemegang Pendaftaran Produk
10. Tarikh kemaskini RiMUP

Apakah kegunaan Arcoxia tablet

Doktor telah mempreskripsikan Arcoxia untuk:

- merawat osteoarthritis
- merawat reumatoid arthritis
- merawat spondilitis ankilos
- sakit pinggang kronik (30 mg dan 60 mg sahaja)
- merawat sakit akut selepas prosedur pergigian minor
- merawat senggugut
- merawat serangan gout

Osteoarthritis ialah sejenis penyakit sendi. Osteoarthritis berlaku akibat kemusnahan beransur-ansur pada rawan yang melindungi hujung- hujung tulang lalu mengakibatkan rasa sakit, radang, rasa sakit apabila bergerak, kekakuan dan ketidakupayaan fizikal.

Arthritis reumatoid ialah sejenis penyakit kronik yang menyebabkan rasa sakit, kekakuan, bengkak dan hilang fungsi pada sendi serta radang pada organ-organ yang lain di

dalam tubuh.

Spondilitis ankilos ialah sejenis penyakit radang tulang belakang dan sendi yang besar.

Gout ialah sejenis penyakit yang dicirikan oleh serangan rasa sakit dan radang yang berulang-ulang dan berlaku secara tiba-tiba pada satu sendi atau lebih.

Anda boleh dapatkan maklumat selanjutnya daripada doktor ataupun daripada ahli farmasi kerana mereka ada maklumat yang lebih terperinci.

Bagaimana Arcoxia tablet berfungsi

Arcoxia terangkum dalam kumpulan ubat yang digelar perencat khusus COX-2 yang digunakan untuk mengurangkan rasa sakit dan radang. Arcoxia bukan bahan narkotik.

Sebelum mengambil Arcoxia tablet

- Bila tidak boleh mengambil
Jangan makan Arcoxia tablet jika anda:

- alah terhadap mana-mana bahannya
- pernah mengalami kegagalan jantung, serangan jantung, pembedahan pintasan, sakit dada, arteri yang sempit atau tersumbat pada hujung-hujung anggota, strok atau strok kecil
- tekanan darah tinggi yang belum dikawal dengan rawatan

- Sebelum mula mengambil
Beritahulah doktor tentang apa-apa masalah kesihatan atau alergi yang pernah atau yang sedang

anda alami, termasuklah:

- sejarah berlakunya angina, serangan jantung atau arteri jantung yang tersumbat
- arteri yang sempit atau tersumbat pada hujung-hujung anggota
- penyakit ginjal (buah pinggang)
- penyakit hati
- dehidrasi, misalnya akibat muntah-muntah atau cirit-birit yang berpanjangan sejarah pendarahan dari perut atau ulser di dalam perut
- kegagalan jantung
- tekanan darah tinggi
- bengkak akibat penakungan bendalir tubuh
- tindak balas alergi terhadap aspirin atau ubat antiradang yang lain (lazimnya digelar NSAID).
- sejarah berlakunya strok atau strok kecil
- keadaan yang meningkatkan risiko anda mendapat penyakit arteri koronari atau aterosklerosis seperti tekanan darah tinggi, diabetes, aras kolesterol tinggi atau tabiat merokok.
- rawatan jangkitan yang sedang anda jalani. Arcoxia tablet boleh menyembunyikan demam, iaitu suatu tanda jangkitan.
- mendapat asma (lelah), radang hidung, dan/atau gatal-gatal atau gegata kerana menggunakan Aspirin atau NSAID.
- dispepsia (ketakcernaan).

Etoricoxib boleh menyebabkan tindak balas yang teruk pada kulit. Antara simptomnya ialah kulit yang beruam, melepuh dan mengelupas. Ini mungkin tanda-tanda penyakit yang

serius. Jika tindak balas begini berlaku, berhenti menggunakan ubat ini dan dapatkan rawatan perubatan dengan segera.

Penggunaan oleh kanak-kanak

Arcoxia tablet belum lagi dikaji pada kanak-kanak dengan secukupnya. Oleh itu, Arcoxia tablet tidak boleh diberikan kepada kanak-kanak.

Penggunaan oleh warga tua

Arcoxia tablet sama baik keberkesananannya pada pesakit dewasa yang tua mahupun yang muda. Kesan sampingan mungkin lebih kerap berlaku dalam kalangan pesakit yang lebih tua berbanding dengan pesakit yang muda. Jika anda warga tua (iaitu berusia 65 tahun ke atas), doktor akan mengawasi anda dengan sewajarnya. Dos tidak perlu diubah bagi pesakit yang tua.

Penggunaan semasa hamil dan semasa menyusukan anak dengan susu badan

Beritahu doktor jika anda:

- sedang atau bercadang hendak menyusukan anak dengan susu badan. Tablet Arcoxia boleh memudaratkan janin dan tidak boleh digunakan oleh wanita yang sedang cuba menjadi hamil ataupun yang telah menjadi hamil semasa masih menerima rawatan.
- Sedang atau merancang untuk menyusukan anak dengan susu badan.

- Jika mengambil ubat-ubat lain

Anda mestilah memberitahu doktor tentang semua ubat yang anda makan atau yang anda bercadang hendak makan, termasuklah ubat yang anda peroleh tanpa preskripsi.

Doktor boleh memastikan bahawa ubat anda bertindak dengan baik jika anda menggunakan:

- *warfarin* (ubat pencair darah)

- *rifampin* (sejenis antibiotik)
- bahan diuretik (tablet yang menyurutkan cecair badan)
- perencat ACE dan penyekat reseptor angiotensin (ubat-ubat yang digunakan untuk merawat tekanan darah tinggikan kegagalan jantung)
- *litium* (sejenis ubat yang digunakan untuk merawat sejenis kemurungan)
- ubat pencegah kehamilan
- terapi penggantian hormon
- methotrexate (sejenis ubat yang digunakan untuk menyekat sistem imun).

Arcoxia tablet boleh dimakan bersama dengan aspirin dos rendah. Jika anda sedang memakan aspirin dos rendah untuk mencegah serangan jantung atau strok, jangan berhenti memakannya tanpa menanya doktor kerana Arcoxia tablet tidak boleh menggantikan aspirin bagi tujuan ini.

Cara mengambil Arcoxia tablet

- Berapa banyak harus diambil

Arcoxia tablet hendaklah dimakan sekali sehari. Arcoxia tablet boleh dimakan dengan atau tanpa makanan.

Doktor akan menentukan dos Arcoxia tablet yang harus anda makan dan tempohnya.

Untuk merawat osteoarthritis:

- * Dos yang disyorkan ialah 30mg atau 60 mg sekali sehari.

Untuk merawat reumatoid arthritis:

- * Dos yang disyorkan ialah 60 mg sekali sehari dan boleh dinaikkan ke 90mg sekali sehari jika perlu.

Untuk merawat spondilitis ankylos:

- * Dos yang disyorkan ialah 60 mg sekali sehari dan

boleh dinaikkan ke 90 mg sekali sehari jika perlu.

Untuk sakit pinggang kronik:

- * Dos yang disyorkan ialah 60 mg sekali sehari.

Untuk merawat rasa sakit yang akut

Dos yang disyorkan ialah 90mg atau 120mg sekali sehari. Arcoxia hanya sepatutnya digunakan selama tempoh kesakitan akut itu, tanpa melebihi 8 hari.

Untuk melegakan serangan gout dan untuk merawat senggugut:

- * Dos yang disyorkan ialah 120mg sekali sehari dan sepatutnya digunakan hanya semasa tempoh rasa sakit yang akut.

Untuk melegakan rasa sakit selepas pesakit menjalani prosedur pergigian yang kecil

Dos yang disyorkan ialah 90mg sekali sehari.

Dos yang melebihi dos yang disyorkan bagi setiap penyakit sama ada tidak menunjukkan keberkesanan yang lebih tinggi ataupun belum lagi dikaji. Oleh itu, jangan makan melebihi dos harian yang dinyatakan bagi setiap penyakit di atas ini.

Jika anda menghidap penyakit hati yang ringan atau sederhana teruk, doktor mungkin memberi anda preskripsi yang lebih rendah ataupun menasihatkan agar anda tidak menggunakan Arcoxia.

- Bila perlu diambil

Ikut arahan doktor atau ahli farmasi anda.

- Berapa lama perlu digunakan

Teruskan penggunaan ubat ini untuk tempoh masa yang ditetapkan oleh doktor anda.

- Jika terlupa mengambil

Cuba makan Arcoxia tablet seperti yang dipreskripsikan. Walau bagaimanapun, jika anda terlupa memakan satu dos, jangan makan dos tambahan. Teruskan sahaja jadual anda seperti biasa pada keesokan hari.

- Jika mengambil berlebihan (terlebih dos)

Jika anda termakan dos yang melebihi preskripsi, hubungilah doktor dengan segera.

Semasa mengambil Arcoxia tablet**- Perkara yang perlu dilakukan**
Hubungilah doktor anda jika:

- Mana-mana simptom yang berikut timbul atau bertambah teruk: sesak nafas, sakit dada atau bengkak pada pergelangan kaki. Hentikan rawatan anda dengan Arcoxia tablet dan jumpalah doktor dengan secepat mungkin.
- Anda menghidap penyakit ginjal, hati atau jantung, doktor akan mengawasi anda dengan sewajarnya.
- Anda mengalami apa-apa simptom yang mungkin disebabkan oleh tindak balas alergi yang teruk, misalnya kesukaran bernafas atau tindak balas kulit yang serius, anda mestilah berjumpa doktor dengan segera.

Doktor akan berbincang tentang rawatan anda dari semasa ke semasa. Penting agar anda menggunakan dos terendah yang boleh mengawal rasa sakit anda.

Arcoxia tablet boleh meninggikan tekanan darah pada sesetengah orang, terutamanya dengan dos yang tinggi, dan ini boleh meningkatkan risiko mendapat serangan jantung dan strok.

Doktor akan memeriksa tekanan darah anda dari semasa ke semasa untuk memastikan bahawa rawatan masih selamat diteruskan.

- Perkara yang tidak boleh dilakukan

Jangan kongsi Arcoxia tablet dengan orang lain kerana ubat ini dipreskripsikan hanya untuk anda khususnya.

Jangan makan Arcoxia tablet lebih lama daripada perlu. Ini kerana risiko mendapat serangan jantung dan strok mungkin akan meningkat selepas tempoh rawatan yang panjang, terutama sekali dengan dos yang tinggi.

- Perkara yang perlu diperhatikan

Berhati-hati apabila memandu atau mengendalikan jentera sehinggalah anda tahu kesan Arcoxia terhadap anda.

Arcoxia mengandungi laktosa. Jika doktor memberitahu anda bahawa anda tidak dapat menahan sesetengah jenis gula, hubungilah doktor sebelum anda mula menggunakan produk perubatan ini.

Kesan-kesan sampingan

Mana-mana ubat pun boleh memberikan kesan yang tidak dimaksudkan atau yang tidak diingini, yang digelar kesan sampingan. Sama seperti semua ubat preskripsi yang lain, Arcoxia tablet juga boleh menyebabkan kesan sampingan.

Antara kesan sampingan yang dilaporkan dalam kajian ialah:

- pening,
- bengkak pada betis dan/atau kaki,
- lemah badan dan lesu,
- tekanan darah tinggi,
- loya,

- pedih hulu hati,
- sakit perut,
- sakit kepala
- soket gigi yang kering (radang dan sakit selepas gigi dicabut)

Selain itu, kesan sampingan yang berikut pernah dilaporkan:

- tindak balas alergi (alahan) (yang mungkin cukup serius sehingga perlu segera dirawat) termasuk bengkak pada muka, bibir, lidah, dan/atau tekak, yang boleh menyukarkan pesakit bernafas atau menelan, nafas berdehit, ruam, gatal-gatal dan gegata (kelopak garam);
- kulit menjadi merah-merah,
- tindak balas kulit yang teruk, yang mungkin berlaku tanpa amaran;
- perubahan pada rasa lidah,
- ritma jantung yang abnormal,
- kegagalan jantung,
- palpitasi (debaran jantung),
- dada berasa sempit, tertekan atau berat,
- sakit perut,
- ulser di dalam perut yang boleh menjadi serius dan berdarah, dan boleh berlaku pada bila-bila masa apabila menggunakannya, tanpa amaran,
- muntah-muntah.
- masalah hati termasuk kegagalan hati, kulit dan mata menjadi kuning (jaundis),
- masalah ginjal (buah pinggang) yang serius,
- aras kalium yang tinggi di dalam darah,
- insomnia (susah tidur),
- keresahan,
- kemurungan,
- keresahan,
- mengantuk,
- ulser di dalam mulut,
- cirit-birit,
- peningkatan tekanan darah yang teruk,

- keliru,
- halusinasi,
- platelet berkurang,
- penglihatan kabur,
- gumpalan darah dalam saluran darah paru-paru dan / atau kaki,
- pendarahan dalam otak.

Dalam kajian klinikal, risiko pengguna Arcoxia tablet mendapatulser lebih rendah berbanding dengan pengguna NSAID.

Kesan sampingan yang lain jugaboleh sekali-sekala berlaku, dansama seperti mana-mana ubat preskripsi yang lain, sesetengah daripadanya mungkin serius. Tanyalah doktor atau ahli farmasi untuk mendapatkan maklumat selanjutnya. Mereka ada senarai kesan sampingan yang lebih lengkap. Beritahulah doktor atau ahli farmasi dengan segera tentanggejala ini atau tentang apa-apa gejala lain yang luar biasa.

Anda boleh melaporkan sebarang kesan sampingan atau kesan adversubat kepada Pusat Pemantauan Kesan Advers Ubat Kebangsaan melalui laman web npra.gov.my [Consumers ➔ Reporting Side Effects to Medicines (ConSERF) or Vaccines (AEFI)]

Cara penyimpanan dan pelupusan Arcoxia tablet

- Penyimpanan

Simpanlah pada suhu yang kurang daripada 30 °C (86 °F). Simpan di dalam bungkusannya yang asal. Jauhkan daripada lembapan dancahaya.

Simpan Arcoxia dan semua

ubatdengan selamat jauh daripada kanak-kanak.

Jangan makan ubat ini selepas bulan dan tahun yang ditunjukkan oleh empat angka selepas EX (atauEXP) yang tertulis pada bekas.

Dua angka pangkal menunjukkan bulan, manakala dua angka hujungmenunjukkan tahun.

- Pelupusan

Ubat-ubatan tidak harus dilupuskan melalui air buangan atau sisa isi rumah. Sila rujuk kepada ahli farmasi andamengenaicara yang betul untuk melupuskanubat yang tidak diperlukan lagi.

Lagnkah-langkah ini dapat membantu melindungi aam sekitar.

Maklumat lanjut

- Rupa dan warna produk

- Arcoxia 30mg: Tablet bersalut berwarna biru muda-hijau dan berbentuk epal dwicembung dengan ACX 30 terukir di sebelah tablet dan 101 di sebelah yang lain.
- Arcoxia 60mg: Tablet bersalutberwarna hijau gelap dan berbentuk epal dwicembung dengan Arcoxia 60 terukir di sebelah tablet dan 200 di sebelah yang lain.
- Arcoxia 90mg: Tablet bersalutyang berwarna putih dan berbentuk epal dwicembung dengan Arcoxia 90 terukir di sebelah tablet dan 202 di sebelah yang lain.
- Arcoxia 120 mg: Tablet bersalut berwarna hijau pucat dan berbentuk epal dwicembung dengan Arcoxia 120 terukir di sebelah tablet dan 204 di sebelah yang lain.

- Bahan-bahan kandungan

- Bahan aktif:

Arcoxia (etoricoxib) ialah tablet yang mengandungi 30, 60, 90, atau 120 mg etoricoxib sebagai ramuannya yang aktif.

- Bahan tidak aktif:

Calcium hydrogen phosphate, anhydrous; microcrystalline cellulose; croscarmellose sodium; hypromellose; magnesium stearate; lactose monohydrate; titanium dioxide; glycerol triacetate dan carnauba wax. Tablet 30 mg, 60 mg dan 120 mg juga mengandungi yellow ferric oxide and FD&C Blue #2 (indigo carmine lake).

- Nombor MAL

Arcoxia 30 mg tablet: MAL12065063ACRZ

Arcoxia 60 mg tablet: MAL12065082ACRZ

Arcoxia 90 mg tablet: MAL12065065ACRZ

Arcoxia 120 mg tablet: MAL12065084ACRZ

Pengilang

Rovi Pharma Industrial Services, S.A
Vía Complutense, 140 28805
Alcalá de Henares Madrid, Spain

Pemegang Pendaftaran Produk

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Tarikh kemaskini RiMUP

11/10/2023 (versi 012023)

Nombor siri

NPRA (R1/2) 02012024/222
NPRA (R1/AI1) 24052023/012